From: Hui, Jennie
To: Doug Robb; Kevin Murray
Subject: RE: Action Required: Minimum Metadata & Study Selection for HeSANDA DOI Registration
Date: Wednesday, 23 September 2026 11:52:48 AM
Attachments: data dictionary bhas-vars.xlsx
Data dictionary bbbs-vars1.xlsx

OFFICIAL

Hi Doug,

Apologies for the delay in getting back to you on this. Please see my answer below in red.

Regards,
Jennie

From: Doug Robb <doug.robb@curtin.edu.au>
Sent: Thursday, 17 September 2026 1:05 PM
To: Hui, Jennie <Jennie.Hui@health.wa.gov.au>; Kevin Murray <Kevin.Murray@uwa.edu.au>
Subject: RE: Action Required: Minimum Metadata & Study Selection for HeSANDA DOI Registration

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Report Suspicious

Hi Jennie and Murray,

On the DOI granularity question our rule of thumb is that if a dataset has its own data dictionary and can stand alone for secondary use, it gets its own DOI. If it's just a part of a bigger dataset it can be described within the one DOI. Remember these are just to describe catalogue entries and not downloadable datasets so a DOI is just a place holder.

One thing I'd like to flag before we get too deep into the DOI question, though is for our side of the process, the RAiD is the priority. That's the **project level** container we need registered first regardless of how the DOI granularity evolves.

I am pre-populating a spreadsheet to send back for checking but before I do this can you answer these questions.
For each of BHAS, BBBS and BHS24:

1. **Official title** — the full formal name of the study (you've already got the acronyms; just need the confirmed full name attached to each).

BHS – Busselton Health Study
BHAS – Phase 1: Busselton Healthy Ageing Study (2011-2015)
BBBS – Phase 2: Busselton Baby Boomer Study (2016-2022)
BHS24 – Phase 3: Busselton Healthy Ageing Study Phase 3 (2024-2028)

2. **Start date** (and end date, if it's concluded/closed to new recruitment).
3. **A few sentences describing the study** — what it is, who it covers, what it's for. Doesn't need to be polished, I can flesh out against the website or if there are publications.

The Busselton Healthy Ageing (Baby Boomer) Study (BHAS/BBBS):
Phase 1: 2010-2015 (Study code: BHAS)
Baseline assessment, cohort n= 5107, adults b.1946-1964.
Phase 2: 2016-2022 (Study code: BBBS)
Second assessment of BHAS Cohort, n = 3888.
Phase 3: 2024-2028 (Study code: BHS24)
Third assessment of BHAS Cohort.

4. **Who's the project leader, and who else should be listed as a contributor** — just full names and their rough role (e.g. "CI/leader," "co-investigator," "data manager"). I'll match these to ORCIDs once I have the names.

Alan James – Chair, CI
Jennie Hui – Director, CI, biospecimen custodian
Michael Hunter – Director, CI
Kevin Murray – Chair of Research Committee, CI, data custodian

5. **Who's the right day-to-day contact** for questions about the data (name + email) — this might be the same person as #4 or someone else entirely.

Kevin Murray (kevin.murray@uwa.edu.au) will be the best person for questions about data, but I am happy to be listed as other day-to-day contact if it is helpful (jennie.hui@uwa.edu.au).

6. **Lead organisation** — which institution owns/hosts the study, UWA? (I'll find the ROR).

Busselton Population Medical Research Institute Inc

7. Send me the data dictionary, and confirm the data is available for secondary

use?

See attached data dictionary for Phase 1 and Phase 2.

Phase 3 data collection is currently underway so not yet available.

This will let me create the RAiDs (project level container) and we can work through the rest later.

Thanks,
Doug

From: Hui, Jennie <Jennie.Hui@health.wa.gov.au>
Sent: Wednesday, 26 August 2026 9:29 AM
To: Doug Robb <doug.robb@curtin.edu.au>; Kevin Murray <Kevin.Murray@uwa.edu.au>
Subject: RE: Action Required: Minimum Metadata & Study Selection for HeSANDA DOI Registration

OFFICIAL

Hi Doug,

With regards to your question about studies that we need to create a DOI for, my initial thought is that we should assign separate DOIs to each "wave" (e.g. DOI A for BHAS, DOI B for BBBS and DOI C for BHS24) to make it easier to identify and track usage of specific datasets. We do have separate data dictionaries for BHAS and BBBS.

Having said that, I am wondering if you have any examples on how other longitudinal cohorts are handling this? Are they using a just a single DOI for the whole cohort with multiple timepoints and what their rationale behind it?

Regards,
Jennie

From: Doug Robb <doug.robb@curtin.edu.au>
Sent: Friday, 14 August 2026 2:48 PM
To: Hui, Jennie <Jennie.Hui@health.wa.gov.au>; Kevin Murray <Kevin.Murray@uwa.edu.au>
Subject: Action Required: Minimum Metadata & Study Selection for HeSANDA DOI Registration

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Hi Jennie and Kevin,

I hope this email finds you well.

This is just a heads up that we are now at a stage in the Health Studies Australian Data Asset (HeSANDA) cohorts project where I need to assemble the metadata for the studies you wish to include in the data catalogue. There is a minimum metadata requirement to generate DataCite Digital Object Identifiers (DOIs), which are used in the Health Data Australia (HDA) catalogue.

The BPMRI website provides a good high-level overview of your studies, but the DataCite schema requires specific, structured items to register a DOI.

At a minimum, to create a DOI we need:

- **Specific Study Identifiers:** A definitive list of distinct study titles, creators, and organisations (e.g., organisation UWA, ORCID of researchers/contributors) of datasets you intend to contribute to the HeSANDA catalogue.
- **Creators and Contributors:** The exact names, institutional affiliations, and (if available) ORCID IDs of the primary authors, chief investigators, or data custodians related to each study.
- **Data Availability & Structure:** Confirmation of which specific datasets or waves are available for access under the HeSANDA framework. This is why the data dictionary is a key aspect of the catalogue.

I can see that there are Excel downloads of data dictionaries for many of your surveys. This will mean picking through these and either creating a DOI for each (which would be the best option for discoverability) or grouping them as related items to a base study. I will work through this later, but for now, could I get a list of studies that you can identify we will need to create a DOI for?

For example:

- Could the surveys from 1966–1987 be lumped together under one single DOI?
- Should the Healthy Ageing Study be treated as one study with multiple timepoints?

Please note that if studies do not have data dictionaries, or if the data is not available for secondary use, they cannot be included in the catalogue. I will need to filter what goes in and what stays out, so that information will be incredibly helpful to have upfront.

Additionally, I assume, Kevin, that you will be the designated contact person for the data access requests?

To make this step as easy as possible, I have attached a simple three-column spreadsheet template.

You can use this to create worksheets for each study DOI with title, map their creators, and confirm if a data dictionary is ready. The **Resource Type General** is already locked in as

Dataset to align with the catalogue's focus.

I am in Melbourne next week to meet with the other nodes and compare progress, so if I can at least identify which studies (e.g., the Busselton Healthy Ageing Study and the Busselton Respiratory Study) will be suitable, I can swing back to you after those meetings.

The catalogue is not set up around using institute directors as creators so this is something I will nut out next week (if the study was funded by an NHMRC or MRFF grant, I can generally extract that background information myself from the grant records).

Looking forward to hearing from you.

Thanks,

Doug.

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